Nursing Essentials

Priority: Early-pregnancy pain, bleeding, or collapse requires assessment for hemorrhage, including an ectopic pregnancy.

FOCUS 1

Recognize

- **Possible findings:** unilateral pelvic pain, vaginal bleeding, or an abnormal pregnancy evaluation.
- **Rupture warning:** severe pain, shoulder-tip pain, dizziness, syncope, hypotension, or shock.
- **Hidden loss:** substantial internal bleeding may occur with little vaginal bleeding.

FOCUS 2

Stabilize

- **Unstable client:** activate urgent care, assess perfusion, establish access, and prepare for resuscitation and surgery.
- **Diagnostic workup:** ultrasound and serial quantitative hCG are interpreted together; one hCG result does not locate the pregnancy.
- **Stable treatment:** medication or surgery depends on eligibility and the clinical plan. Serial follow-up is essential.

FOCUS 3

Follow hCG

- **Possible clues:** bleeding, unusually high hCG, severe nausea, or a uterus larger than expected; ultrasound helps establish the diagnosis.
- **Early preeclampsia:** hypertension with concerning findings before the usual timeframe deserves investigation.
- **After treatment:** serial hCG monitoring checks for persistent gestational trophoblastic disease.

Teach after treatment

Make it stick: Falling hCG does not make new severe pain safe to ignore. Clinical instability always overrides reassurance from a prior laboratory trend.

Focus	What to know	Nursing application
Methotrexate	follow the prescriber's medication, alcohol, folate, activity, and pregnancy-avoidance instructions.	Follow the specific plan
Return immediately	severe pain, fainting, heavy bleeding, or other symptoms of rupture / hemorrhage.	Explain return precautions
Molar follow-up	use the advised contraception plan; a new pregnancy can make hCG surveillance difficult to interpret.	Protect follow-up

RED FLAGS · ESCALATE PROMPTLY

Severe abdominal or shoulder-tip pain, fainting, hypotension, or heavy bleeding may indicate rupture or hemorrhage—even with little visible blood.